



PAWSIC

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

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STANDARDS & ASSESSMENT FRAMEWORK

2026 EDITION

Mobile Wound Group *Checklist*

A standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings.

SECTION IV • SETTING-SPECIFIC ADD-ON

Assisted Living / Independent Living *ALF/IL*

Complete in addition to Section I: Core Standards.

What's *Inside*

- ✓ Limited Clinical Infrastructure ALF-1
- ✓ Coordination with External Providers ALF-2
- ✓ Medication & Treatment Oversight ALF-3
- ✓ Risk Management ALF-4
- ✓ Family / Responsible Party Communication ALF-5

Scoring

- 0 Absent
- 1 Emerging
- 2 Established
- 3 Best-in-Class

★ Critical items are flagged throughout to signal elevated regulatory or clinical risk.

HOW TO USE

Complete Section I, then your *setting add-on*

Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV).

SECTION I
Core

SECTION II
SNF

SECTION III
HHA

SECTION IV
ALF/IL

Purpose

The PAWSIC Mobile Wound Group (MWG) Checklist 2026 is a standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings. It is intended to support:

- Wound Providers (WPs) and Wound Provider Groups (WPGs): A tool to support program development or process improvement.
- Healthcare Leaders: A tool for any post-acute site of service to aid in the selection of a WP or WPG.
- Patients/Responsible Parties: A reference guide when selecting a WP or WPG.

Intent

This checklist is solely a guide and covers a variety of competencies for both employed WPs and contracted WPGs. It is intended to be adaptable to the needs specific to a site of service, regulations, and applicable practice acts. Regulations, reimbursement structures, and policies that direct practice and care differ from state to state, payer to payer, and by site of service.

This checklist is not intended to replace your facility/organization's policies and procedures nor any local, state, or federal requirements. The utilization of this form is at your discretion. PAWSIC releases this form to your facility/organization to assist in selecting or guiding a partnership with a WP/WPG.

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Instructions: Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV). Section V is a patient-facing document; clinical reviewers do not need to complete it.

★ **Critical items are flagged throughout the document to signal elevated regulatory or clinical risk requiring priority follow-up**

Assessment Intake

Complete this block before starting each evaluation. All fields are required for traceability.

FIELD	ENTRY	ADDITIONAL INFO
Facility / Organization Name (include prior company names, acquisitions, rebranding etc)		
WP / WPG Being Evaluated		
Reason for Review: (New group, annual review, post-incident, due diligence, etc.)		
WPG/WP patient volume/footprint (number of facilities/states)		
Reviewer Name, Title/Role		
Reviewer Disclosures (MWG competitor, vendor, former employee)		
Reviewer of Evidence Name, Title/Role		
Date of Assessment		
Care Setting (select one)	☐ SNF (Skilled Nursing Facility) ☐ HHA (Home Health Agency) ☐ ALF (Assisted Living) ☐ IL (Independent Living)	

SECTION IV

Setting-Specific Add-On: ALF/IL

Assisted Living / Independent Living | Complete in addition to all Core sections (Section I) | Skip if care setting is SNF or HHA

SCORING

0 Absent

1 Emerging

2 Established

3 Best-in-Class

★ Critical Items, these need to be summarized separately

Comments/Findings/Actions: Reviewer to complete

Evidence Reviewed: in some cases this may be N/A; in others, the evidence used to support findings should be validated through observation of a legal document, website review, etc.

ALF-1 Limited Clinical Infrastructure

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
How care is adapted when nursing presence is limited or not available on site			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Non-clinical staff involvement: scope, training, and limitations			
Clear escalation protocols for non-clinical caregiving staff			
Defined triggers for escalation to a higher level of care			

ALF-2 Coordination with External Providers

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Communication with primary care providers			
Coordination with home health agencies			
Coordination with hospice providers			
Defined responsibility boundaries between WPG, ALF staff, and external providers			

ALF-3 Medication & Treatment Oversight

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Process for managing dressing orders in a limited-license setting			
Process for medication changes related to wound care			
Staff education for non-clinical caregivers performing dressing changes			
Documentation practices when licensed nurse oversight is intermittent			
Acute wound response pathway is defined or agreed upon This includes clear instructions for initial basic wound care (control bleeding, cleanse, and protect), staff guidance, and defined escalation criteria based on severity. The provider must specify when to notify the wound team, involve urgent care or activate EMS)			

ALF-4 Risk Management

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Fall risk and wound interaction assessment and management			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
State-specific scope-of-practice variation for ALFs between WPs and staff: what staff in this state are legally permitted to do (med-pass, dressing changes) and what can be performed on-site by WP vs out-patient setting?			
If memory care unit, how will consent, WP care, wound monitoring, and family communications work?			
Coordination with resident's PCP or attending			
Infection monitoring approach with limited clinical oversight			
Criteria and process for escalating to a higher level of care			
Skin breakdown prevention program and WPG's role in it			

ALF-5 Family / Responsible Party Communication

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Communication expectations with family and responsible parties			
Consent process for procedures in the ALF/IL setting ★ Critical			
Goal-of-care discussions: how conducted, documented, and communicated			
Process when responsible party disagrees with wound care recommendations			



Post-Acute Wound and Skin Integrity Council (PAWSIC) is a non-profit professional group advocating for best practices in wound care and patient-centered care in the post-acute medical sector.

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